# Website Service Line Content — CCM, PCM, RPM, RTM, TCM

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Ready-to-use website content for Zenith's five Medicare care management service lines. Copy each section onto its own service page, adjusting bracketed placeholders ([Zenith], [state], [practice]) as needed. Reimbursement figures are 2026 Medicare national averages and will vary by locality — confirm current rates before publishing exact dollar figures.

## How to Structure This on the Website

Give each service its own dedicated page under a "Services" parent page, rather than cramming all five onto one page — providers searching for "RPM company" or "CCM billing partner" should land on content specific to that term.

1. **Services overview page** — one-paragraph intro to Zenith + a 5-card grid, one card per service (icon, name, one-line description, "Learn more" button)
2. **Five individual service pages** — one per program (CCM, PCM, RPM, RTM, TCM), each following the same template: what it is → who qualifies → revenue opportunity → what Zenith handles → call-to-action
3. **Comparison/FAQ section** (optional, can live on the overview page) — addresses "what's the difference between CCM and PCM" type questions, which providers genuinely ask and which helps SEO

## CCM — Chronic Care Management

**Page headline:** Chronic Care Management (CCM) — Recurring Revenue for Your Highest-Need Patients

**Opening paragraph:** Chronic Care Management is a Medicare-covered program for patients living with two or more chronic conditions — diabetes, hypertension, COPD, CKD, depression, and more — expected to last 12 months or longer. [Zenith] delivers the monthly care coordination calls, documentation, and medication management these patients need, so your practice earns recurring Medicare reimbursement without adding staff workload.

**Who qualifies:**

- 2 or more chronic conditions expected to last 12+ months
- Enrolled in Medicare Part B
- Documented patient consent on file

**What's billed:**

- CPT 99490 — first 20 minutes/month of clinical staff time (~$62)
- CPT 99439 — each additional 20 minutes (~$47)
- CPT 99487/99489 — Complex CCM, 60+ minutes with high medical decision-making
- CPT 99491/99437 — when time is personally performed by the physician

**What [Zenith] handles:**

- Monthly care coordination calls with each enrolled patient
- Comprehensive care plan creation and updates
- Medication reconciliation and management
- Coordination with specialists and outside providers
- Full documentation inside your existing EHR or CCM platform

**Revenue framing:** A 200-patient CCM panel billing the base code alone generates roughly $13,200/month in Medicare reimbursement for your practice.

**Call to action:** Book a 15-minute discovery call to see how many of your patients qualify.

## PCM — Principal Care Management

**Page headline:** Principal Care Management (PCM) — Focused Support for Your Highest-Risk Single-Condition Patients

**Opening paragraph:** Principal Care Management is a Medicare program built for patients managed around one serious, high-risk chronic condition — such as heart failure, advanced CKD, or severe COPD — expected to last at least 3 months. It's a natural fit for specialty practices where a single condition dominates the clinical picture. [Zenith] runs the monthly management work so your practice captures this reimbursement.

**Who qualifies:**

- One complex, high-risk chronic condition (not two or more — that's CCM)
- Condition expected to last at least 3 months
- Enrolled in Medicare Part B

**What's billed:**

- CPT 99424 — first 30 minutes/month, physician or QHP time (~$88)
- CPT 99425 — each additional 30 minutes, physician/QHP (~$61)
- CPT 99426 — first 30 minutes/month, clinical staff under supervision (~$68)
- CPT 99427 — each additional 30 minutes, clinical staff (~$54)

**Important:** PCM and CCM cannot be billed for the same patient in the same month by the same provider — a specialist can bill PCM for one condition while the patient's PCP separately bills CCM for other conditions.

**What [Zenith] handles:**

- Monthly focused care management for the qualifying condition
- Care plan development centered on that single condition
- Coordination between specialist and primary care team
- Full documentation and time tracking

**Call to action:** Find out which of your patients are a fit for PCM instead of CCM.

## RPM — Remote Patient Monitoring

**Page headline:** Remote Patient Monitoring (RPM) — Turn Connected Devices Into Recurring Revenue

**Opening paragraph:** Remote Patient Monitoring uses connected devices — blood pressure cuffs, glucometers, pulse oximeters, weight scales — to track a patient's physiological data between visits. [Zenith] manages the day-to-day monitoring, patient check-ins, and documentation, turning passive device data into a billable, high-value service for your practice.

**Who qualifies:**

- At least one acute or chronic condition warranting monitoring (hypertension, diabetes, heart failure, COPD, obesity, post-surgical recovery)
- Established relationship with the ordering provider
- Written patient consent on file

**What's billed (2026 codes):**

- CPT 99453 — initial device setup and patient education (~$20, billed once per device)
- CPT 99454 — device supply, 16+ days of data in 30 days (~$52/month)

- CPT 99445 *(new for 2026)* — device supply, 2–15 days of data (~$52/month) — opens RPM to shorter-engagement patients
- CPT 99457 — first 20 minutes/month of monitoring and management (~$52)
- CPT 99458 — each additional 20 minutes
- CPT 99470 *(new for 2026)* — 10–19 minutes of management time, a lower threshold option

**What [Zenith] handles:**

- Reviewing incoming device data for concerning trends
- Monthly interactive check-ins with each patient
- Escalating out-of-range readings to the care team
- Full time tracking and documentation

**Revenue framing:** A single enrolled RPM patient can generate $150–$200+ per month in combined device and management reimbursement.

**Call to action:** Ask us which of your patients already have, or would benefit from, a connected monitoring device.

## RTM — Remote Therapeutic Monitoring

**Page headline:** Remote Therapeutic Monitoring (RTM) — Monitor Therapy Adherence and Response Remotely

**Opening paragraph:** Remote Therapeutic Monitoring tracks non-physiological data — respiratory status, musculoskeletal function, pain levels, and therapy adherence — using digital apps or software, not just hardware devices. It's billable by physicians, NPs, PAs, and also physical/occupational/speech therapists. [Zenith] manages the monitoring workflow and patient engagement so this becomes a reliable revenue line.

**Who qualifies:**

- Patients needing respiratory or musculoskeletal condition monitoring, pain management, or therapy adherence tracking
- Use of an FDA-defined medical device or software as a medical device (SaMD)
- Documented consent on file

**What's billed (2026 codes):**

- CPT 98975 — initial setup and patient education (~$22)

- CPT 98976 — respiratory monitoring device supply, 16–30 days (~$47/month)
- CPT 98977 — musculoskeletal monitoring device supply, 16–30 days (~$40/month)
- CPT 98984/98985 *(new for 2026)* — short-window device supply, 2–15 days — covers shorter-engagement patients
- CPT 98980 — first 20 minutes/month of treatment management (requires a live interactive check-in)
- CPT 98981 — each additional 20 minutes
- CPT 98979 *(new for 2026)* — 10–19 minutes of management time, lower threshold option

**Key difference from RPM:** RPM tracks physiological vitals (blood pressure, glucose); RTM tracks therapeutic/functional data (pain, range of motion, adherence) and can be billed by therapists, not just physicians and mid-levels.

**What [Zenith] handles:**

- Reviewing therapy adherence and functional status data
- Monthly live check-ins (required for the management codes)
- Documentation and escalation of concerning trends

**Call to action:** Find out if RTM fits your patient population alongside or instead of RPM.

## TCM — Transitional Care Management

**Page headline:** Transitional Care Management (TCM) — Close the Gap After Hospital Discharge

**Opening paragraph:** Transitional Care Management covers the critical 30 days after a patient leaves the hospital, observation stay, or skilled nursing facility. Patients who don't receive structured post-discharge follow-up are far more likely to be readmitted — and TCM is one of Medicare's highest-paying care management codes per episode. [Zenith] handles the time-sensitive outreach and coordination this requires.

**Who qualifies:**

- Recently discharged from an acute care hospital, inpatient psychiatric unit, skilled nursing facility, or long-term care hospital
- Returning to the community (home, assisted living, etc.)

**What's billed:**

- CPT 99495 — moderate complexity, face-to-face visit within 14 days of discharge (~$220)
- CPT 99496 — high complexity, face-to-face visit within 7 days of discharge (~$299)
- Only one TCM code is billed per patient per 30-day period

**Strict timing requirements:**

- Interactive contact (phone, video, or in-person) with the patient or caregiver within **2 business days** of discharge — miss this window and the episode isn't billable at all
- Face-to-face visit within 7 or 14 days depending on complexity
- Medication reconciliation during the service period

**What [Zenith] handles:**

- The critical 2-business-day post-discharge outreach call
- Medication reconciliation support
- Coordination with home health, specialists, and community services
- Tracking the visit window and documentation to prevent missed billing

**Call to action:** Ask us how many discharge-eligible patients your practice may currently be missing.

## Side-by-Side Comparison (Internal Reference)

| Program | Eligibility | Primary CPT Codes | Base Reimbursement | Who Can Bill |
|---|---|---|---|---|
| CCM | 2+ chronic conditions, 12+ months | 99490, 99439, 99487/99489, 99491 | ~$62/month | Physician, NP, PA, clinical staff |
| PCM | 1 high-risk condition, 3+ months | 99424, 99425, 99426, 99427 | ~$68–$88/month | Physician, NP, PA, clinical staff |
| RPM | Acute/chronic condition, connected device | 99453, 99454, 99445, 99457, 99458, 99470 | ~$150–$200+/month | Physician, NP, PA |

| Program | Eligibility | Primary CPT Codes | Base Reimbursement | Who Can Bill |
|---|---|---|---|---|
| RTM | Respiratory/MSK condition, app or device | 98975, 98976/98977, 98984/98985, 98980, 98981, 98979 | ~$90–$150/month | Physician, NP, PA, PT, OT, SLP |
| TCM | Post-discharge, 30-day window | 99495, 99496 | $220–$299 per episode | Physician, NP, PA |

**Mutual exclusivity to note on the site or in sales conversations:** CCM and PCM cannot be billed together for the same patient in the same month by the same provider. TCM cannot overlap with CCM billing in the same calendar month as the TCM service period. RPM and RTM use separate code families and generally can be billed concurrently if clinically distinct data is being tracked — confirm current payer-specific rules before combining.

## Suggested Services Overview Page Copy

**Headline:** Five Ways We Help Your Practice Capture Unbilled Medicare Revenue

**Subheadline:** [Zenith] runs the care coordination, monitoring, and documentation work behind five Medicare care management programs — so your practice earns recurring revenue without adding staff.

**Five-card summary (one line each, for the overview grid):**

- **CCM** — For patients with 2+ chronic conditions needing ongoing monthly coordination
- **PCM** — For patients managed around one serious, high-risk condition
- **RPM** — Turn connected device data into monthly recurring revenue
- **RTM** — Monitor therapy adherence and functional status remotely
- **TCM** — Close the gap in the critical 30 days after hospital discharge

**Closing call-to-action section:** Not sure which programs fit your patient panel? Book a 15-minute call and we'll map out exactly which of your patients qualify for each program — and what it could mean for your practice's monthly revenue.